Emergency Department Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

Case ID	SYN-0013	**Age / Sex**	66 / M
Setting	Emergency Dept / Neuro-ICU	**Primary dx**	Hypertensive basal ganglia hemorrhage

Chief complaint

Sudden headache, vomiting, and left-sided weakness.

History of present illness

While gardening he suddenly complained of severe headache, vomited, and fell. Wife found him unable to move his left side with slurred speech. EMS arrival at 25 min. Became drowsier on transport.

Past history / medications

Poorly controlled hypertension (stopped medication 6 months ago), heavy smoker. No anticoagulants or antiplatelets.

Examination

BP 224/128, HR 70, RR 16. Left hemiplegia, left facial droop, right gaze deviation. Pupils 3 mm, equal and reactive. Neck supple.

Glasgow Coma Scale

Eyes open only to painful stimulus. Speaks inappropriate words, no sentences. Motor: localizes pain with the right arm.

GCS on arrival	Score	Response
Eye opening (E)	2	To pain
Verbal (V)	3	3 - Inappropriate words
Motor (M)	5	Localizes pain
TOTAL GCS		**10**

Investigations

CT head: right putaminal hemorrhage, volume 48 mL, with intraventricular extension and midline shift 5 mm. CTA: no aneurysm or AVM, no spot sign. Platelets and INR normal. ICH score 3.

Assessment

Spontaneous hypertensive intracerebral hemorrhage (right basal ganglia) with intraventricular extension. GCS 10.

Plan / disposition

IV nicardipine to target systolic 130-140 mmHg, hyperosmolar therapy, elevate head of bed. Neurosurgical review: external ventricular drain for hydrocephalus risk, no evacuation at this stage. Admitted to neuro-ICU.